THE WHITE ARMY
CASE PRESENTATION
CHRONIC DACRYOCYSTITIS

-AISHWARYA H K

7 TH TERM

BGS GIMS

BANGALORE

- Name – Mrs. XYZ
- Age – 40 years
- Gender – Female
- Education – 10 th standard
- Occupation – home maker
- Address-Behind Poorna Prajna College, 5th cross, JPnagar , Mysore, 570-001
- Socio economic status –Upper lower class (Kuppuswamy scale)
- DOA - 1/7/2020
- DOE – 2/7/2020

•Chief complaints

- 1) Watering in the Left eye since 6 months

History of presenting illness

- Patient was apparently normal 6 months back, later she noticed watering in the left eye , insidious in onset.

- There is no history of discharge
- There is no history of excoriation over the cheek skin.
- There is no other swellings found.
- There is no history of pain.
- There is no history of rhinitis, sinusitis
- There is no history of Itching
- There is no history of redness
- There is no history of trauma
- There is no history of Fever

- **Ocular history –**

There is no history of surgery to the left eye.

- **Past history –**

There is no history of similar complaints in the past.

Patient is a known case of Diabetes Mellitus on regular medication (Metformin)

Patient is a known case of Hypertension on regular medication (Amlodipine)

No other chronic illnesses.

- **Family history –** No history of similar illness.
- **Personal history -**

1. Diet –Predominantly vegetarian.
2. Appetite – Normal
3. Bowel and Bladder movements –Regular
4. Sleep – undisturbed
5. Addictions –not present
6. Drug allergies – not present

· Summary

This is a case of 40 Year old lady , who is an home maker, a known case of DM and HTN, presented with the complaints of watering in left eye since 6 months.

•General physical examination

42 year old lady, who is moderatly built and well nourished is conscious, cooperative, alert, oriented to time, place and person.

Weight – 70 Kg

Height – 165 cm

BMI-27.3 Kg/m^2.

No pallor, icterus, clubbing, cyanosis ,lypadenopathy, edema.

- **Vitals**.

1. Pulse rate -65 bpm
2. Blood pressure – 128/80 mmHg measured in left arm, sitting position
3. Respiratory rate -15 cycles per minute.
4. Temperature- Afebrile.

- **Systemic examination**.

1. CVS –S1 and S2 heard.
2. Respiratory system –B/L Normal Vesicular breath sounds heard .
3. CNS –No focal neurological deficits.
4. Per abdomen – Soft and non tender.

Ocular examination

1. Head posture – erect
2. Facial symmetry – symmetrical
3. Ocular posture –orthotropic.

Ocular examination	Right eye	Left eye
1. Visual acuity		
• Distance vision	Counting finger ,more than 6 meters	Counting finger, more than 6 meters.
• Near vision	Normal	Normal
2. Eye brows		
• Position.	Normal	Normal
3.Eyelids – position	Normal	Normal And no ectropion or entropion
• Palpebral aperture width	Length -30 mm, height 10mm	Length -30 my, height -10 mm

Ocular examination	Right eye	Left eye
4)Lacrimal aparatus - Lacrimal sac	Normal	Normal.
• Lacrimal puncta	Normal	Increased watering from the eye is seen.
• Regurgitation test	Negative	Purulent discharge.
• Lacrimal gland	Normal	Normal
5) Eyeball – size, position, movements of eyeball, uniocular movements, binocular movements	Normal	Normal
6)Conjunctiva – Palpebral, bulbar conjunctiva and conjunctivaa fornices	Normal	Normal

Ocular examination	Right	Left
7) Cornea – size, shape, surface, sheen, sensations, transparency	Normal	Normal
8) Sclera	Normal	Normal
9) Anterior chamber – depth and contents	Normal	Normal
10 Iris - color and pattern	Brown color, normal pattern	Brown color, normal pattern
11)Pupil -	Central, round, blackish	Central, round, blackish.

Ocular examination	Right eye	Left eye
Pupil -pupillary reactions		
• Direct light reflex	Constriction of pupil present	Constriction of pupil present
• Indirect light reflex	Constriction of pupil on both the sides	Constriction of pupil on both the sides
• Near reflex	Constriction of pupil, convergence of eyeball present	Constriction of pupil, convergence of eyeball present
12)Lens-position , shape, color, transparency	Normal	Normal
14) IOP (by digital tonometry)	Normal	Normal

Differential diagnosis –

1. Chronic Dacryocystitis.
2. Foreign body in NLD.
3. Trauma to NLD or punctum or canaliculi.
4. Nasal polyps.
5. Deviated nasal septum.
6. Turbinate hypertrophy.

Provisional diagnosis

- After taking the history and doing the ocular examination, probable diagnosis can be **Chronic Dacryocystitis of the left eye, chronic suppurative stage.**

Thank you